# Trastuzumab subcutaneous (SC) · metastatic

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, while beneficial and tolerated. Any taxane or endocrine partner follows a separate prescription and monitoring plan.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

### 1 Breathing

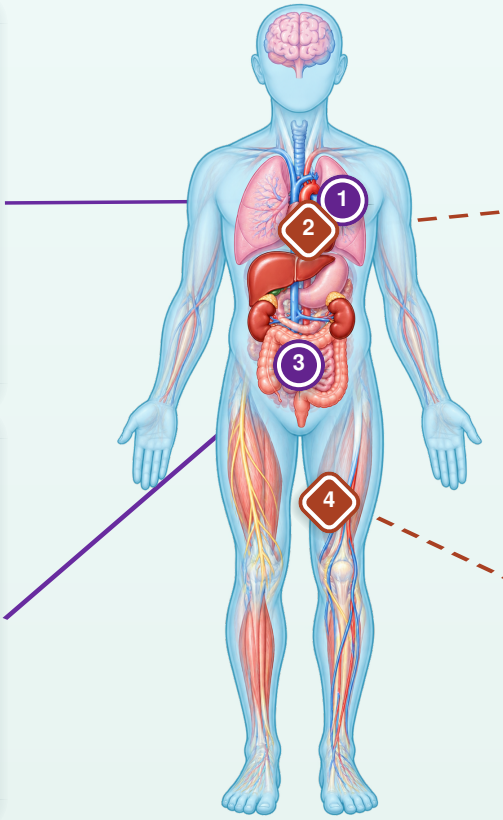
**Trastuzumab**

New cough or breathlessness: seek prompt assessment.

### 2 Heart

**Trastuzumab**

Keep echo checks. Report swelling, chest symptoms, fainting or rapid weight gain.




### 3 Bowel / fatigue

**Trastuzumab**

Report persistent diarrhoea, dehydration or fatigue affecting daily activity.

### 4 Injection site

**SC trastuzumab**

Report painful, spreading redness or swelling where the injection was given.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

# Your treatment, at home.

## Important medicine-specific effects

### Trastuzumab

- Heart pumping-function reduction / heart failure
- Allergic or administration-related reactions
- Injection-site pain, redness or swelling
- Diarrhoea, fatigue and rare serious lung effects

**Surgery, radiation and monitoring:** open the whole-treatment map or scan the QR.

## Make the plan practical

### Support for this medicine

Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.

### Blood and heart appointments

full blood count (FBC), renal and liver profile every 6 weeks (around C3, C5, C7 if on time). Echo / multigated acquisition (heart pumping) scan (MUGA) every 12 weeks; around C5 for uninterrupted 21-day cycles.

### Local treatment and endocrine therapy

Surgery and radiotherapy are coordinated by the multidisciplinary team (MDT). When endocrine therapy is indicated, it has its own schedule and bone-health plan.

### Pregnancy / contraception

Discuss pregnancy, fertility and contraception before treatment. Report pregnancy promptly; do not breastfeed during treatment or for 7 months afterwards. The team confirms the medicine-specific contraception duration.

**Emergency: 112 / 999.** Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

**Contact oncology immediately** for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your out-of-hours pathway / nearest ED**

Other hospitals: use the numbers your team provides.

**Complete reference, visual timeline and passport**

https://sactcheck.com/patient/00272/

Sources and review status are available online. Follow your own prescription and hospital instructions.